

PERSONAS

Tried-and-Burned & Curious-but-Wary

Two working-hypothesis personas for the Bold GLP-1 companion product.

SOURCE	STATUS	AUDIENCE	LAUNCH
2026-05-05 readout (n=4)	Working hypotheses	Medicare-eligible, 65+	Q3 2026 · GLP-1 companion

READ THIS FIRST

These are needs-based archetypes built from a 4-person research sample. They are good enough to target prototypes; they are **not** demographically validated personas. The next research wave — a larger GLP-1 survey plus screener redesign — is the prerequisite to graduate them. The "Honesty" page at the end is non-negotiable reading.

AT A GLANCE

Two starting trust levels, one shared destination.

Both personas converge on the same value props — pill-first GLP-1, guidance over tracking, body composition over scale. What differs is the trust they walk in with.

	PERSONA 01 · TRIED-AND-BURNED	PERSONA 02 · CURIOUS-BUT-WARY
GLP-1 STAGE	Has tried — had a bad experience	Has never tried — has heard the stories
DOMINANT EMOTION	Skepticism	Caution
INTERNAL QUESTION	"Why would this time be any different?"	"I want this to work — but is it safe for me?"
TOP FEAR	Side effects + cost returning	Needles + the horror stories they've heard
FIRST-SCREEN NEED	Proof this is a <i>different</i> path	Trust scaffolding (FDA, Medicare, provider)
WHAT UNLOCKS THEM	Pill option · side-effect management · body-comp proof	Pill option · name brands · provider as safety net
SHARE OF SAMPLE	~3 of 4 (not generalizable)	~1 of 4 (not generalizable)

PERSONA 01 · SKEPTIC

01

Tried-and-Burned

Has direct GLP-1 experience that went poorly. Walked away skeptical. Looking for a credibly *different* path back.

“Last year I did GLP1 uh semiclude for like six weeks. It was the worst experience of my life. Cost me \$400 to lose seven pounds and I felt effects... If it's okay, if I could do a pill, that'd be great. Pills easy. Pen second. Compounded with a vial like that. No way. Definitely no.

PARTICIPANT · 2026-05-05 TRANSCRIPT, 00:12:29

DOMINANT EMOTION

Skepticism. "Why would this time be any different?"

FIRST-30-SECONDS BAR

Show what is *different* — or they bounce.

TOP FEARS

- Side effects (nausea, constipation, headaches) returning.
- Paying again and getting weak results — "\$400 to lose seven pounds."
- Being put back on a shot, pen, or compounded vial.

TOP NEEDS

- A credibly different path — pill-first form factor.
- Active side-effect management with a provider as safety net.
- Cost certainty up front — no surprise bills.
- Proof they're protecting muscle and bone, not just dropping scale weight.

WHAT SUCCESS LOOKS LIKE FOR THEM

They stay on the medication past the 6-week mark where they quit last time, because **the side effects are actively managed**, they can **see they're protecting strength** (not just scale weight), and the **cost is stable and explained**.

IMPLICATIONS FOR DESIGN

1. **Lead with side-effect management and form-factor choice.** Don't bury either behind feature-led copy.
2. **Never** show a vial, syringe, or compounded medication in hero imagery.
3. **Pair every safety signal** (FDA, name brand) with a coverage/cost signal in the same view.
4. **Body-composition proof has to feel low-effort.** A burdensome measurement strategy collapses the value prop.
5. **Acknowledge the prior experience.** "If a GLP-1 didn't work for you before, here's what we do differently" will land harder than a generic welcome.

EVIDENCE Bad-side-effects + bad-results frame: 2026-05-05 transcript, 00:12:29 · Form-factor preference (pill > pen >> injection >> vial): findings.md #2 · \$50/mo against the \$300-\$700 anchor: findings.md #19 · Body composition as differentiator over scale weight: findings.md #6.

PERSONA 02 · CAUTIOUS

Curious-but-Wary

02

Has never tried a GLP-1. Has heard both the success stories and the horror stories. Considering it for the first time as Medicare coverage opens.

I mean, much easier to do the pill for me because I'm definitely afraid of shots. ... As you get older you have to worry about bone density and stuff, but body composition — it stands out more than the rest of the other ones here.

PARTICIPANTS · 2026-05-05 TRANSCRIPT, 00:13:23 & 00:15:42

DOMINANT EMOTION

Caution. "I want this to work — but is it safe for me?"

FIRST-30-SECONDS BAR

Earn first trust — FDA, Medicare, provider, pill.

TOP FEARS

- Needles.
- The side-effect stories they've heard from friends and family.
- Cost, even with Medicare in the picture.
- "Is this even safe for someone my age?"

TOP NEEDS

- Strong trust scaffolding up front: FDA approval, name brands, provider in the loop.
- A gentle entry — the pill option as the default visual frame.
- A clear, early cost story tied to Medicare coverage.
- Reassurance that aging is *protected for*, not ignored — bone density, strength, balance.

WHAT SUCCESS LOOKS LIKE FOR THEM

They feel safe enough to start. The first 30 days don't deliver the horror stories they were told to expect. They have a **real person they can call** when something feels off — but not one who pesters them with daily check-ins.

IMPLICATIONS FOR DESIGN

1. **Onboarding must earn first trust.** FDA + Medicare coverage signals visible in the first screen or two.
2. **Provider-as-safety-net** language, not provider-as-coach. "A clinician will check in on side effects and dosage" — not "Chat with your coach today!"
3. **Pill as default visual frame.** Pen as a clearly-offered secondary. No needle imagery anywhere.
4. **Acknowledge the stories without amplifying them.** "You've probably heard scary stories about GLP-1s. Here's what we do to keep you safe."
5. **Surface body-composition framing early** — many in this group are aging-aware and want to protect strength and bone, not just shrink.

EVIDENCE Fear of shots: 2026-05-05 transcript, 00:13:23 · Body composition + bone density as aging concerns: 2026-05-05 transcript, 00:15:42; findings.md #8 · Trust signals trigger cost questions — pair them: findings.md #4, principles.md #5 · Provider as safety net, not coach: findings.md #5, principles.md #6.

UNIVERSAL CONSTANTS

True for both personas.

Every prototype must respect these regardless of which persona it targets. They are the floor — the place where audience consensus is strong enough to treat as design law.

CONSTANT	SOURCE
Pill > pen >> injection >> compounded vial.	findings.md #2
Guidance, not tracking. Help <i>before</i> the decision, not after.	findings #9–11 · principles #1–2
Real person for accountability + side-effect management — not chat coaching.	findings #5 · principles #6
FDA approval & name brands matter but trigger cost questions — pair them.	findings #4 · principles #5
Body composition (lean mass, bone density, fat-vs-muscle) is the differentiator, not scale weight.	findings #6 · principles #3
Older-adult-appropriate by default — seated options; standing tolerance can be under 5 minutes.	findings #15–16 · principles #7
No wearables, no extra devices.	findings #12 · principles #8
\$50/month, tied to Medicare coverage, surfaced early.	findings #19 · principles #11

HONESTY SECTION

What these personas do *not* cover.

The bullseye sample was deliberately small. Treat every gap below as research debt — work to do before the personas earn the right to anchor anything beyond prototypes.

GAP	WHY IT MATTERS	WHEN IT GETS RESOLVED
Ages	Ages are not disclosed for any participant. We can't tune age-specific copy or imagery with confidence.	Larger survey + screener redesign
Health diagnoses	Pre-diabetic / diabetic / post-menopausal status is not attributed per person. Some recurring themes (restrictive-diet fatigue, bone density worry) likely correlate with diagnoses we can't see.	Screener redesign
Household & eating patterns	Cooking, takeout, spouse roles are real in the transcript but not tied to a persona. The "75% takeout, spouse stopped cooking" quote could belong to either archetype.	Larger survey
Validated proportions	Roughly 3:1 Tried-and-Burned vs Curious-but-Wary in the n=4 sample. Not generalizable to the broader Medicare-age audience.	Larger survey

BOTTOM LINE

Use these personas to target prototype work. Do **not** use them to make brand or marketing-strategy decisions yet. Re-evaluate the moment the larger research lands — and if it contradicts the bullseye sample, this document is the first thing to update.

SOURCES

2026-05-05 · Weight Management Research Readout
Full readout + transcript. data/raw/Weight Management Research Readout – 2026_05_05 14_29 PDT – Notes by Gemini.pdf

Synthesis files (working memory)
data/synthesis/ — findings.md · decisions.md · principles.md · value-props.md · personas.md · positioning.md · open-questions.md

2026-05-04 · Weekly Product/Design Sync
Early readout + Q3 launch commitment. data/raw/Weekly Product_Design Sync – 2026_05_04 11_30 PDT – Notes by Gemini.pdf

Companion document
design-brief.md — the prototype-phase design brief that consumes these personas.